

ختم  
شعار الجمهورية

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|                                                                            |                                                                                     |
|----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> امل رياض عبد الخالق حسن<br><b>National ID:</b> 28901242300401 |  |
|----------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                     |
|-------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging |
|-------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

٢٠٢٤/١١ ٢٨٩٠١٢٤٢٣٠٠٤٠١

رؤية منزل

أنثى مسلمة

محمود أبو السعود بطور نصر

البطاقة سارية حتى ٢٠٣٨/١١/٠١

١

الجمهورية العربية السورية  
بطاقة تحقيق الشخصية

امل  
رياض عبد الخالق حسين

ح ١٤٥٥  
الشيخ زايد - الجيزة

٢٨٩٠١٢٤٢٣٠٠٤٠١

KU7562743

MDT FORM

7/9/20

G

|                   |                |                |             |
|-------------------|----------------|----------------|-------------|
| Name              | امل, راجح فيال | MRN            | 7509        |
| Age               | 36             | ID             |             |
| Menopausal status | premen         | Phone          | 019906719   |
| NRS SCORE         |                | Marital status | CA906322002 |
| BMI               |                | Family history |             |
| CO morbidity      | -ve            | REASON OF MDT  |             |

BX: IDc Clinical Data

|     |     |        |       |
|-----|-----|--------|-------|
| ER: | PR: | HER 2: | Ki67: |
| sls | sls | 2uph   |       |

Sonomammography: Denar met Bc  
to liver. Bm, Bm

Mets Workup: Pt start Taxol, Herceptin  
pt received 36 wk of taxol &  
14 cycles of Herceptin  
Pt develop PAIN GUT

Recommendations:

Last CT cap 4-2-25 → D.R.  
MRI b/w 2-6-25 → Sol's ab 3 to 14mm

MDT OUTCOME & PLAN

Jo Maintenance AI ± 2 levels of Her E2 level  
+ Monitor Herceptin IV

SIGNATURE

|                  |         |           |           |           |             |
|------------------|---------|-----------|-----------|-----------|-------------|
| Medical Oncology | Surgery | Radiology | Pathology | Nutrition | Psychiatric |
|                  |         |           |           |           |             |



|                                                    |                                                     |
|----------------------------------------------------|-----------------------------------------------------|
| Patient Name : الأستاذة / أمل رياض عبد الخالق حسين | Request No. : 13009702                              |
| Patient No. : 28901242300401                       | Request Date : 07/08/2024                           |
| Path No. : SR-1099/24                              | Verification Date : 20/08/2024                      |
| Sex : Female                                       | Financial Category : نفقة الدولة - اورام دار السلام |
| Age : 35 Y                                         | Department : Out Patient                            |
| Referring Doc :                                    | Registered by : nianwa mohamed youss                |

### Pathology Report

Clinical Data: Right breast mass.  
Nature of Specimen: Slide revision.

#### Gross Examination:

- Five paraffin blocks labeled by (840536) and Nine slides were received.

#### Microscopic Examination:

- \* Histological type: Multifocal invasive duct carcinoma.
- \* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- \* DCIS: Positive (90%) (comedo, cribriform, solid)
- \* Angioinvasion: No.
- \* Perineural invasion: No.
- \* IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: No nuclear reaction in the tumor cells (score 0/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

#### Diagnosis:

Right breast cancer, Slide revision:  
Multifocal invasive duct carcinoma, Grade II with DCIS.

#### IPT:

- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- For further IPT (SISH).

Signature

Verified by :

Dr. Sagda El-Kady

Printing Date : 17/12/2024 10:38:06

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



|                 |                                      |                    |                                  |
|-----------------|--------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذ / أمل رياض عبد الخالق حسين | Request No.        | : 13009702                       |
| Patient No.     | : 28901242300401                     | Request Date       | : 07/08/2024                     |
| Path No.        | : SR-1099/24                         | Verification Date  | : 20/08/2024                     |
| Sex             | : Female                             | Financial Category | : أورام دار السلام - نفقة الدولة |
| Age             | : 35 Y                               | Department         | : Out Patient                    |
| Referring Doc : |                                      | Registered by      | : .marwa mohamed youss           |

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#### Diagnosis:

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- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- For further IPT (SISH).

Signature

Verified by :

Dr. Sagda El-Kady

Printing Date : 17/12/2024 10:38:06

Page 1 of 2

Printing By : Mrs. Mayada Hegazy





Patient Name : الأستاذة / أمل رياض عبد الخالق حسين  
Patient No. : 28901242300401  
Path No. : SR-1099/24  
Sex : Female  
Age : 35 Y  
Referring Doc :

Request No. : 13009702  
Request Date : 07/08/2024  
Verification Date : 20/08/2024  
Financial Category : نفقة الدولة - اورام دار السلام  
Department : Out Patient  
Registered by : .marwa mohamed youss

### Pathology Report

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- For further IPT (SISH).

Signature

Verified by :

Dr. Sagda El-Kady



|                                                 |                   |
|-------------------------------------------------|-------------------|
| Patient Name: Amaal Reyad Abd Al Khalek Hussein | Date : 10/05/2025 |
| Exam : C.T CHEST WITH CONTRAST                  | Age : 36 Year(s)  |
| Medical ID: 2509/2024                           | Pat. ID: 77262    |

**POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

**➤ Known case of right BCS . \*\*As compared to the last study 23-01-2025, The current revealed:**

**1. Chest:**

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Stable course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 1x0.5 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Stationary** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, the largest is noted at inferior lingular segment still measuring 4 mm.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

**2. Abdomen and pelvis:**

- The liver is of average size with **rather stationary course regarding number and size of** bilobar discrete and confluent hypodense focal lesions, The largest at hepatic dome measuring currently about 1.3 x1 cm.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

|                                                 |                   |
|-------------------------------------------------|-------------------|
| Patient Name: Amaal Reyad Abd Al Khalek Hussein | Date : 22/07/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis)          | Age : 36 Year(s)  |
| Medical ID:                                     | Pat. ID: 77262    |

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
  - No pathological abdominal or pelvic lymph nodes.
  - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
  - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
  - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
  - Currently noted bilateral adnexal cystic lesions for sonographic assessment.
  - Normal appearance of the other pelvic organs.
  - No abdominal or pelvic masses or collections.
- 3. The scanned skeleton:**
- Sacralized L5 vertebra. Stationary course of D4, L1 vertebral body and left iliac wing lytic lesions measuring 7 mm. Small D5 sclerotic lesion. 1st degree lytic spondylolisthesis of L4.
  - No vertebral structural collapse or extra-osseous soft tissue component.
- OPINION:**
- Reported case of right BCS SHOWING***
- Overall Regressive course since the last study.

Dr/Aya Gabr

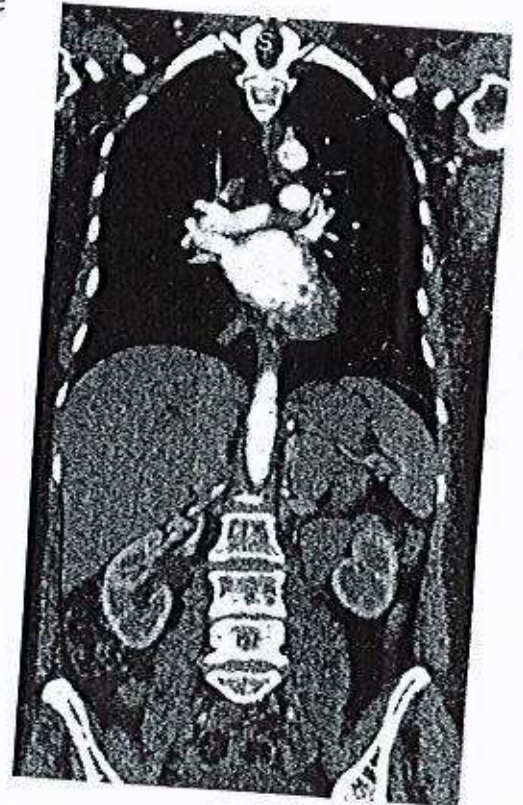
|                                                 |                   |
|-------------------------------------------------|-------------------|
| Patient Name: Amaal Reyad Abd Al Khalek Hussein | Date : 22/07/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis)          | Age : 36 Year(s)  |
| Medical ID:                                     | Pat. ID: 77262    |

**POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

➤ **Known case of right BCS . \*\*As compared to the last study 10-05-2025, The current revealed:**

**1. Chest:**

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Regressive course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 0.5x0.3 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Regressive** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, seen less appreciated in the current study.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



**2. Abdomen and pelvis:**

- The liver is of average size with **Regressive course regarding number and size of** bilobar discrete hypodense focal lesions, seen less appreciated in the current study
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

Patient Name: Amal Riad Abd El-Khalek Hussein  
Prof. Dr: Clinics Masr El-Hadetha  
Accession: 240903134

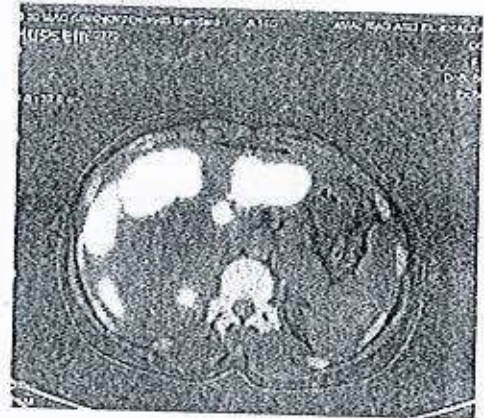
Patient ID: 684364  
Exam Date: 3/9/2024

### III) Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- No metabolically active (FDG avid) pleural effusion is noted.
- No FDG avid (metabolically active) mediastinal lymph-nodes are noted.
- Normal cardiac size with no pericardial effusion.

### IV) Abdomen and pelvis:-

- Mildly enlarged liver showing few metabolically active ill-defined bilobar hypodense hepatic focal lesions measuring up to 7 x 5.6 cm with max SUV ~ 12.77 (at hepatic dome lesion).
- Metabolically active few enlarged aortocaval, portocaval and porta hepatis lymph nodes are noted measuring up to 3 cm with max SUV ~ 10.45 (at porta hepatis hepatic lymph node).
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No ascites.
- Metabolically active right adnexal cyst is noted measuring 2.7 cm with max SUV ~ 7.18 (corpus luteum).
- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.



Name : Amal Reyad Abdel Khalek Hussein

Date : 10-02-2026 10:56

Branch : Portable

Medical No. : GRI-77262

Ref. Physician Prof : CT CAP+C

Ins. Company :

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

➤ Known case of right BCS . Compared to the last study 18/11/2025, The current revealed:

**1. Chest:**

- **No appreciable time interval changes** regarding to the size of the few right axillary lymph nodes became prominent the largest measures about 1x0.5 cm.
- Rather stationary course regarding the previously seen few tiny bilateral scattered pulmonary nodules largest measuring 3.5 mm in diameter at the left lower lobe.
- **No appreciable time interval changes regarding the number of the previously seen few bilateral scattered faint ground glass densities.**
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

**2. Abdomen and pelvis:**

- The liver is of average size *with regressive course regarding the previously seen few bilobar* hypodense focal lesions, The largest at segment VIII measuring currently about 6 mm in diameter in today's study.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Regressive course mounting to complete resolution regarding the previously seen right ovarian cyst.
- Normal appearance of the other pelvic organs.
- Regressive course mounting to complete resolution regarding the previously seen mild pelvic collection.

**3. The scanned skeleton:**

Name : Amal Reyad abdel khalek hussei

Branch : Portable

Ref.Physician Prof : CT CAP+C

Date : 10-02-2026 10:56

Medical No. : GRI-77262

Ins.Company :

- No appreciable time interval changes regarding the previously seen few vertebral and left iliac wing lytic as well as the D5 sclerotic focus.
- Still seen 1<sup>st</sup> degree lytic spondylolisthesis of L4.
- No vertebral structural collapse or extra-osseous soft tissue component.

**OPINION:**

***Reported case of right BCS SHOWING***

- Regressive course regarding the:
  - The size and number of the previously seen few bilobar hypodense hepatic focal lesions.
  - The previously seen right ovarian cyst no longer seen in today's study.
  - The previously seen mild pelvic collection.
- Stationary course regarding the rest of the study.

*Dr. Sherif Wagdy*

*Dr. Sherif Wagdy*

Name : Amaal ryad abd elkhalik hussin

Branch : Portable

Ref.Physician Prof : brain +c

Date : 10-02-2026 11:20

Medical No. : GRI-

260210111719STD131221107523748902

Ins.Company :

## **MRI OF THE BRAIN**

### **Technique :**

Axial T1, T2 ,FLAIR, DWI& ADC

Coronal T2

Sagittal T2

Postcontrast T1

### **Findings: compared to last study dated 21-6-2025 ;**

- Marked regressive course as regard the size and shape of the previously noted multiple bilateral cerebral SOL seen at both frontal , parietal basal ganglia and , both thalami and right subependymal and left cerebellar region , the lesion currently seen as foci and small areas of altered MR signal eliciting low T1 and bright T2 and FLAIR W1 signal with contrast uptake, some of them show minimal surrounding edema, no significant mass effect ,the largest one is seen at right lateral ventricle sub endymal location measuring about 6mm compared to lareset one 14 mm in the las study with regression of the previously noted marked perifocal edema .
- Normal size and configuration of the supra and infra tentorial ventricular system
- No midline shift or deformity
- Normal cortical sulci
- No cerebello pontine angle masses,
- Partial empty sella.
- Prominent peri-optic CSF spaces bilaterally.

### **OPINION:**

- *Marked regressive course of the previously noted cerebral and cerebellar intra axila SOLs compared to last study , for follow up and clinical correlation.*